

Daily Content Intel

May 28, 2026

Topic A — Reel

TELEPROMPTER COPY (paste into Instagram)

Okay, this is the part of athletic development almost no one actually trains. Deceleration.

About 70% of non-contact ACL tears happen in the braking step, the cut, or the hard plant. Almost all of them happen while the athlete is trying to slow down.

When you decelerate, the ground reaction force spikes high and fast, what the researchers call a tall, thin force curve, and your knee, your hip, and your tendons have to absorb all of that in milliseconds.

If you've never trained for that load, the tissue absorbs it poorly. The knee

collapses inward, the hip stays high, and the joint takes a hit it was never prepared for.

Here's the fix. Expose the athlete to deceleration on purpose, every week. Short maximal sprints into a controlled stop, on grass, on turf, in the directions they actually play.

In my clinic, when I bring a post-op ACL athlete back to sport, the last clearance test is braking under load, well after the squat and the box jump are cleared.

The CROSS framework calls this teaching the body to absorb force before it produces it.

If you skip it, you're training a faster engine with no brakes.

Be Good. Help Someone. Learn Lots.

Hook: 70% of ACL tears happen when an athlete tries to stop.

Spoken script (word for word):

Okay, this is the part of athletic development almost no one actually trains. Deceleration.

About 70% of non-contact ACL tears happen in the braking step, the cut, or the hard plant. Almost all of them happen while the athlete is trying to slow down.

When you decelerate, the ground reaction force spikes high and fast, what the researchers call a tall, thin force curve, and your knee, your hip, and your tendons have to absorb all of that in milliseconds.

If you've never trained for that load, the tissue absorbs it poorly. The knee collapses inward, the hip stays high, and the joint takes a hit it was never prepared for.

Here's the fix. Expose the athlete to deceleration on purpose, every week. Short maximal sprints into a controlled stop, on grass, on turf, in the directions they actually play.

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and the box jump are cleared.

The CROSS framework calls this teaching the body to absorb force before it produces it.

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On-screen text seeds:

- 0:00 70% of ACL tears = deceleration
- 0:08 The stop, the cut, the plant
- 0:18 Tall-thin force curve = high impact, short time
- 0:30 Untrained tissue absorbs it poorly
- 0:42 Train the brake on purpose, every week
- 0:55 Last clearance test post-op: braking under load
- 1:05 CROSS: absorb force before you produce it
- 1:15 Faster engine, no brakes

Caption (copy-paste ready):

70% of non-contact ACL tears happen in the stop, not the sprint.

If your training program doesn't expose the athlete to maximal-intensity deceleration on purpose, every week, you're building a car with no brakes.

In my clinic, the last clearance test for a post-op ACL athlete is braking under load. Long after the squat and the box jump are cleared.

The CROSS framework teaches the body to absorb force before it produces it. Train the brake.

Dr. Lars Stevenson, PT, DPT It's time to cross your threshold.

threshold.clientsecure.me

#PhysicalTherapy #ACL

#AthleticDevelopment

#StrengthAndConditioning

#DecelerationTraining #SportsMedicine

#ReturnToSport #InjuryPrevention

#FootballTraining #SoccerTraining

Personalize this

- Which post-op ACL athlete from this year is the cleanest example of "cleared on strength tests but not on braking" — I can name them in voiceover and add a clip of the failed deceleration test
- What's my exact protocol for first-week deceleration exposure in the clinic (distance, stop zone, surface, reps) — drop the prescription into the on-screen text so it's not just theory
- How do I explain the tall-thin force curve to a parent in 1 sentence — that's the line I want to land when I look back at the camera

Screenshots:

- title | <https://pmc.ncbi.nlm.nih.gov/articles/PMC8761154/> | "Deceleration Training in Team Sports: Another Potential 'Vaccine' for Sports-Related Injury?"
- physiology | <https://pmc.ncbi.nlm.nih.gov/articles/PMC8761154/> | "The braking steps during deceleration have a unique ground reaction force profile characterised by a high impact peak force in a shorter time frame"

- actionable | <https://pmc.ncbi.nlm.nih.gov/articles/PMC8761154/> | "Team sport athletes may need to be regularly exposed to high-intensity deceleration within their weekly microcycle"

Sources:

- <https://pmc.ncbi.nlm.nih.gov/articles/PMC8761154/>
- <https://pmc.ncbi.nlm.nih.gov/articles/PMC12273744/>

Topic B — Reel

TELEPROMPTER COPY (paste into Instagram)

Okay, here's a conversation I keep having in clinic. Active women in their 40s and 50s assume that because they run, cycle, swim, and lift, menopause is going to glide right past them.

A new PLOS One paper from December says otherwise.

187 female endurance athletes, ages 40 to 60. 88% had sleep problems. 83% reported physical and mental exhaustion. 72% had anxiety. 67% reported weight gain. And 63% had joint and muscular discomfort.

Here's the part that matters for me as a PT. Of the women who had joint and muscle pain, 97% said it hurt their training, and 91% said it hurt their performance.

That joint pain you've been chalking up to your training load probably has a second driver. Estrogen has dropped, and connective tissue handles stress differently when it does.

So here's my hot take. I'm done pretending fitness is a vaccine against menopause. The data is clear.

In my clinic, the women crushing it through perimenopause are the ones lifting hard, sleeping like it's part of training, and getting a real medical workup on their hormones. The ones piling more endurance volume on top of a body in transition show up with tendons that won't recover and joints that ache for weeks.

If you're a coach, ask the question. If you're the athlete, advocate for the workup. Stop trying to out-train it.

Be Good. Help Someone. Learn Lots.

Hook: Being a fit endurance athlete will not save you from menopause.

Spoken script (word for word):

Okay, here's a conversation I keep having in clinic. Active women in their 40s and 50s assume that because they run, cycle, swim, and lift, menopause is going to glide right past them.

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If you're a coach, ask the question. If you're the athlete, advocate for the workup. Stop trying to out-train it.

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On-screen text seeds:

- 0:00 Fitness $\neq$ a menopause vaccine
- 0:10 187 female endurance athletes, 40-60
- 0:18 88% sleep problems
- 0:22 83% mental + physical exhaustion
- 0:26 63% joint + muscle pain

- 0:35 Of those with joint pain: 97% said it hurt training
- 0:50 Estrogen drop changes how connective tissue handles stress
- 1:05 Lift hard. Sleep hard. Workup the hormones.
- 1:18 Stop trying to out-train it.

Caption (copy-paste ready):

Being a fit endurance athlete will not save you from menopause.

New PLOS One study, December 2025. 187 female endurance athletes, ages 40 to 60.

88% sleep problems. 83% exhaustion. 63% joint and muscular discomfort. And of the women with that joint and muscle pain, 97% said it was actively hurting their training.

That ache you've been chalking up to training load probably has a second driver. Estrogen has dropped, and your connective tissue handles stress differently when it does.

The women I see crushing it through perimenopause are the ones lifting hard,

sleeping like it's part of training, and getting a real medical workup on their hormones.

Stop trying to out-train it.

Dr. Lars Stevenson, PT, DPT It's time to cross your threshold.

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#Menopause #Perimenopause

#WomensHealth #FemaleAthlete

#PhysicalTherapy #EnduranceTraining

#StrengthTraining #JointPain

#HormoneHealth #ActiveWomen

Personalize this

- Name a specific perimenopausal patient who came in with "tendinitis" that turned out to be the estrogen story — I want to anchor the abstract data in a real Threshold case
- What's the exact lift-sleep-hormone-workup conversation I have on day one with that patient — give me the 3 questions so I can read them off camera
- Where do I send these patients for the medical workup locally (Reston) —

naming the referral channel makes the CTA real and not just "talk to your doctor"

Screenshots:

- title | <https://journals.plos.org/plosone/article?id=10.1371/journal.pone.0335738> | "Frequency and perceived influence of menopausal symptoms on training and performance in female endurance athletes"
- physiology | <https://pmc.ncbi.nlm.nih.gov/articles/PMC12711079/> | "The most commonly reported menopausal symptoms were sleep problems (88%), physical and mental exhaustion (83%)"
- actionable | <https://pmc.ncbi.nlm.nih.gov/articles/PMC12711079/> | "Clinicians should address these symptoms to promote continued participation in physical activity"

Sources:

- <https://journals.plos.org/plosone/article?id=10.1371/journal.pone.0335738>
- <https://pmc.ncbi.nlm.nih.gov/articles/PMC12711079/>

Reference

Runner-up topics

- Chris Chase (Just Fly Ep. 515) on tendon resilience and plyometric progression — wide-base teaching reel
- Sports Med 2025-2026 review on horizontal deceleration assessment methodology — coach-facing
- Brazilian American football injury epidemiology paper (pre-activity routines, sleep, mood as risk factors) — possible hot-take on warmup theater

Sources scanned today

- Newsletter digest (2026-05-27): Mercola headlines (sensationalist, not citable); Mike T. Nelson podcast w/ Tracy Holemeyer (nervous-system somatics, too soft for a research reel)
- PubMed: PT/rehab RCTs (mostly stroke telerehab, not relevant to Lars's audience)
- NSCA / JSCR May 2026 issue: military foot march meta-analysis (paywalled, off-audience)

- Sports Medicine / Springer: deceleration & ACL injury research (strong hit)
- PLOS One December 2025: menopause symptoms in female endurance athletes (strong hit, on-audience)
- Iron Culture (May): bodybuilding/nutrition focused, not today's lane
- Just Fly Performance Podcast Ep. 515 (May 14, Chris Chase on plyometrics/deceleration/tendons): convergent signal, pointed me toward the deceleration angle
- Examine / Peter Attia / Huberman: no fresh standout this week
- Reddit r/AdvancedFitness, r/physicaltherapy: nothing above the noise floor